
VITAL ATHLETIC RECOVERY

The Staff Guide.

Modality benefits & usage.

How we talk about every recovery and movement modality with members — with confidence, clarity, and care.

INTERNAL — STAFF REFERENCE
Edition 1 · East Dallas

■■ WELCOME

How to use *this* guide.

This is your reference for every recovery and movement modality we offer — what it does, who it's for, how to talk about it, and how to keep members safe.

Every chapter is built the same way so you know exactly where to look. Use it during onboarding, before your shift, or in the middle of a member conversation when you need a quick reminder.

Read it cover to cover the first week, then keep it nearby. Refresh on a modality before recommending it. You don't need to memorize the science — you need to know it well enough to be a guide, not a salesperson.

EACH CHAPTER INCLUDES

- **What it is** — the modality in plain language.
- **The science** — just enough to sound credible without bluffing.
- **What members get** — the felt benefits, ranked by what people notice first.
- **How to talk about it** — the language that works, and the language that doesn't.
- **Recommended duration & cadence** — what to suggest.
- **Safety & contraindications** — when to slow down or refer out.
- **Common member questions** — answered concisely, in our voice.
- **Pairings** — what works well alongside it in a session.

■■ FOUNDATIONS

The *Vital* voice.

How we speak shapes how a member feels. Read these once — then carry them into every conversation.

WHAT WE SOUND LIKE

- **Calm, not clinical.** We use real science, but we translate it. If a member needs jargon, we go there. Otherwise, plain language wins.
- **Grounded, not hype-y.** No “life-changing,” no “miracle.” The studio’s quiet confidence does the work.
- **Curious about the member, not the script.** We ask before we recommend.
- **Honest about discomfort.** Cold plunge is hard. Sauna is hot. We don’t pretend otherwise — we walk people through it.
- **Inclusive of the body in front of us.** We don’t assume an athlete or a beginner. We meet people where they are.

WHAT WE NEVER DO

- Make medical claims (“this will cure” / “this will heal”). Recovery *supports* the body — that’s the verb.
- Compare ourselves to another studio by name.
- Push the most expensive option. The right tier for the member is always the right tier.
- Rush. The pace of the studio is part of the product.

THREE PHRASES TO REACH FOR

- “The first few minutes are the hardest. Most people are surprised by minute three.”
- “Start where it feels manageable — not where you think you should.”
- “What does your week usually look like?” (Asked before recommending cadence.)

■ FOUNDATIONS

Reading *the member*.

The first 30 seconds of a member's visit tells you most of what you need to know. You're not solving a problem — you're noticing what they're carrying.

THREE THINGS TO LISTEN FOR

- **The state of their nervous system.** Talking fast, holding their phone, jaw tight? Lead with the infrared sauna (with red light), not cold.
- **What they came in for.** “Sore from a run” vs. “long week” are different sessions.
- **What they're avoiding.** If they haven't tried cold yet, don't sell it. Show them where it is and let them decide.

THREE THINGS TO ASK

- “Have you done anything like this before?”
- “Is there anything that's tight or bothering you today?”
- “Do you have a time you need to be done by?”

DEFAULT FIRST-VISIT SEQUENCE

If the member doesn't have a preference: 15 min infrared sauna with red light → 20 min compression → 1 min cold plunge → 5 min traditional sauna finish. Forty-five minutes, low intensity, gives them a taste of four modalities. Adjust based on what you heard.

■ FOUNDATIONS

The recovery *protocol*.

Members ask “what should I do today?” more than any other question. These are the four sequences to know by heart.

01 · THE RESET

Audience: stressed, tight, mentally cooked. The most common request.

Infrared sauna with red light 30 min → compression 20 min (with breathing). Total ~50 min. No cold.

02 · THE ATHLETE

Audience: post-training soreness, sport-specific recovery, run/lift days.

Myofascial 15 min → compression 25 min → traditional sauna 10 min → cold plunge 2 min → brief sauna finish 5 min. Total ~60 min.

03 · THE CONTRAST

Audience: experienced members who want the Nordic cycle for clarity and mood.

Traditional sauna 12 min → cold plunge 1–2 min → rest 3 min. Repeat 2–3 rounds. Finish on heat. Total ~45 min.

04 · THE SHARPENING

Audience: needs to be on for a meeting, presentation, hard workout. Stimulating, not draining.

Cold plunge 1–2 min → infrared sauna with red light 12 min → brief traditional sauna finish 5 min. Total ~25 min. Sends them out alert.

WHEN IN DOUBT, FINISH ON HEAT

Ending a session in the sauna leaves members feeling calm and warm — the “afterglow” they remember. Ending on cold leaves some people wired and chilled. Default to heat last, unless the member specifically wants the sharpening effect.

■ ■ PART TWO

The Modalities.

Seven tools. One member at a time. Each chapter is a deeper look at how to guide a member through the modality with confidence.

NO.	MODALITY	SHORT NAME
01	Cold Plunge	The clarifier
02	Infrared Sauna + Red Light	The gentle reset
03	Traditional Sauna	The original recovery tool
04	Compression Therapy	The passive flush
05	Myofascial Therapy	The unwinder
06	Reformer Classes	The controlled strength
07	Yoga & MindBody	The missing piece

01

MODALITY

Cold Plunge.

Targeted cold exposure — the sharpest, most direct lever we offer.

WHAT IT IS

A short, intentional immersion in cold water (typically 50°F / 10°C) for one to three minutes. Members step in to the chest, breathe through the cold-shock response, and step out. That's the whole thing.

THE SCIENCE (THE CREDIBLE VERSION)

Cold water triggers a sympathetic nervous system spike — a controlled stress dose. Norepinephrine rises sharply (200–500% in the first minute), which is what drives the clarity and mood effects. Cold also constricts peripheral blood vessels and, on rewarming, the body flushes nutrient-rich blood back through tissue. Long-term cold exposure activates brown adipose tissue and is linked in research to improved metabolic markers and stress resilience.

WHAT MEMBERS GET

- **Sharper focus and mood.** Most members describe this first.
- **Lower perceived stress.** Members who plunge regularly handle high-pressure days better.
- **Reduced inflammation.** Best after intense exercise or for nagging soreness.
- **Built-up stress tolerance.** Doing something hard on purpose, daily, generalizes.

HOW TO TALK ABOUT IT

MEMBER: *I don't think I can do this.*

YOU: “Most people say that. You don't have to commit to three minutes — start with thirty seconds. Get in to your waist, then chest. Breathe slow. That's the whole job.”

YOU: “The first ten seconds are the loudest. If you can stay calm through them, the rest is easier than it looks.”

MEMBER: *How cold is it?*

YOU: *“Fifty degrees. That's cold enough to do the work, not so cold that you're white-knuckled. We keep it dialed in.”*

DURATION & CADENCE

First time: 30–60 seconds. Stand, don't crouch. Slow nasal exhales.

Building tolerance: 1–2 minutes, 2–3 times per week.

Experienced: 2–3 minutes, daily. Diminishing returns past 3 minutes per session.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

Pregnancy, cardiovascular disease, uncontrolled high blood pressure, Raynaud's syndrome, recent open wounds. Members on blood-pressure medication should consult their physician. Anyone who feels lightheaded should step out — we never persuade someone to stay in.

COMMON MEMBER QUESTIONS

Will I gasp?

Probably, the first time. That's the cold-shock response — totally normal. The fix is slow nasal exhales. Two or three breaths in, you'll settle.

Before or after a workout?

Depends on the goal. After hard training: cold reduces inflammation but blunts the adaptation signal for hypertrophy. For pure recovery and mood, after is great. For sharpening focus, before is better.

How often?

Two to four times a week is the sweet spot for most people. Daily is fine. Less than once a week and you don't keep the adaptations.

PAIRINGS

Classic contrast: **traditional sauna** → **cold plunge** → **sauna again**. Cold also pairs well at the very start of a session for an alert, focused state, or at the end of a workout for inflammation.

02

MODALITY

Infrared Sauna + Red Light.

Deeper tissue heat plus photobiomodulation — two evidence-backed effects, one cabin.

WHAT IT IS

A wood-paneled cabin with two things inside: infrared emitters that warm the body directly with light wavelengths rather than heating the air, and medical-grade LED panels that emit red (around 660 nm) and near-infrared (around 850 nm) light. Air temperature sits in the 120–150°F range, but tissue heating is comparable to a hotter dry sauna. Members get passive heat conditioning *and* the cellular benefits of red light in a single 25–45 minute session.

THE SCIENCE (THE CREDIBLE VERSION)

The heat side. Infrared light penetrates a few centimeters into tissue and is absorbed by water and tissue molecules, warming the body from the inside. Heart rate rises into a moderate cardio range (110–130 bpm for many users) without external exertion — what researchers call passive heat conditioning. Regular use is associated with improved endothelial function and lower resting blood pressure. Sweating mobilizes some heavy metals and lipophilic toxins, though sweat volumes are lower than traditional sauna.

The red-light side. Red and near-infrared wavelengths are absorbed by cytochrome c oxidase in the mitochondria — the cell's energy-producing organelles. The result is increased ATP production, modulated nitric oxide signaling, and reduced cellular oxidative stress. The clinical literature is strongest for skin (collagen synthesis, wound healing) and joint inflammation, with growing evidence for sleep quality, mood, and recovery from training.

WHAT MEMBERS GET

- **Cardiovascular conditioning** without joint impact.
- **Muscle recovery** — heat increases blood flow through tissue.
- **Skin quality** — texture, firmness, fine lines, and tone over weeks of use.
- **Joint comfort** — especially knees, shoulders, lower back.
- **Better sleep** when used in the evening.
- **Faster recovery** from exercise when used post-training.
- **Easier tolerated** than traditional sauna by people new to heat.

HOW TO TALK ABOUT IT

MEMBER: *What's the difference from a regular sauna?*

YOU: *“Two things. Air temperature is lower — about 140 instead of 190 — so it's more comfortable to breathe. And the heat goes deeper into your tissue, so even at a lower air temp you'll get a real sweat going. Plus there are red-light panels built into the cabin doing different work at the cellular level. It's a softer experience that does more.”*

MEMBER: *Will I actually sweat?*

YOU: *“Yes — about ten minutes in. It's a slower build than a traditional sauna, but it's a deep sweat. Bring water.”*

MEMBER: *I don't feel the red light doing anything.*

YOU: *“Most people don't, and that's normal. This isn't like the heat where the effect is loud — the red-light work happens at the cellular level. Members usually start noticing skin changes around three to four weeks, energy and sleep shifts sooner.”*

DURATION & CADENCE

First time: 20 minutes. Bring water in. Eyes closed when facing the panels.

Building tolerance: 30–35 minutes, three times per week.

Experienced: 40–45 minutes, four to five times per week.

Position for the light: rotate which body areas face the panels across sessions — shoulders one visit, face the next, back another. Swim attire or minimal clothing helps the light reach skin.

SAFETY & WHEN TO SLOW DOWN OR REFER OUT

Heat-related: pregnancy (consult OB), uncontrolled high or low blood pressure, severe cardiovascular disease, dehydration, alcohol use, fever. Stop and exit if dizzy. Hydrate before, during, and after. Older adults and people new to heat should start at the lower end of the duration range. **Red-light-related:** photosensitizing medications (some antibiotics, retinoids, St. John's Wort, certain chemotherapies) — consult the prescriber first. Active skin cancer or lesions of unknown origin should be evaluated by a dermatologist before treatment. Keep eyes closed when facing the panel.

COMMON MEMBER QUESTIONS

Is it safer than traditional sauna?

It's not safer — it's gentler on the airways and easier to sit through. Both have strong safety profiles for healthy adults. Choice comes down to preference.

Will it really help my sleep?

If you use it 60–90 minutes before bed, the post-sauna cool-down mirrors the body's natural pre-sleep temperature drop. A lot of members report falling asleep faster within the first week.

Is the red light safe for my eyes?

Keep eyes closed when facing the panel inside the sauna. Avoid looking directly at the LEDs for extended periods — same as you'd avoid staring at any bright light.

Does the red light tan me?

No — it's red and near-infrared light, not UV. There's no tanning, no burning, no skin damage risk.

Naked or clothed?

Light penetrates skin best with as little fabric as possible. Swim attire is recommended. Comfort decides.

Can I use my phone in there?

Honestly? We'd rather you didn't. Bring water and let yourself check out for half an hour. It's the most underrated part.

PAIRINGS

Excellent **as a session-opener** to warm up tight tissue before myofascial work, or as the **finisher** after cold plunge for the contrast cycle. For a deeper reset, follow with **compression** in the lounge to extend the parasympathetic shift.

03

MODALITY

Traditional Sauna.

Dry heat at 170–185°F — the original, and still the gold standard.

WHAT IT IS

A wood-lined room heated by an electric or wood-burning stove. Air temperature sits between 170 and 185°F. Humidity stays low unless a member ladles water on the stones. Sessions run 10–20 minutes per round, with cooling breaks between rounds.

THE SCIENCE (THE CREDIBLE VERSION)

The traditional sauna has the strongest research base of any modality we offer. The Finnish KIHU study tracked over 2,300 men for 20+ years and found that 4–7 sauna sessions per week were associated with significantly lower cardiovascular mortality and all-cause mortality compared to one session per week. The mechanism is hormetic heat stress: heart rate rises into a moderate-to-vigorous cardio range, heat shock proteins (HSP70) are induced, plasma volume expands, and the cardiovascular system adapts much as it does to exercise.

WHAT MEMBERS GET

- **Real cardiovascular work** without moving.
- **Mental decompression.** The heat is loud; everything else gets quieter.
- **Heat tolerance and resilience** — useful in summer, useful in life.
- **The afterglow.** Members describe the post-sauna calm more than any single benefit.

HOW TO TALK ABOUT IT

MEMBER: *I get out of breath just thinking about it.*

YOU: *“Then start with ten minutes on the lower bench. Heat rises, so the lower you sit, the cooler it is. You can build up to twenty over a few weeks.”*

YOU: *“The first time we usually keep it short on purpose. You leave wanting more, not toasted.”*

MEMBER: *How long should I stay in?*

YOU: *“Listen to your body. For most people the sweet spot is 12 to 18 minutes, then a cool-down, then back in if you want a second round.”*

DURATION & CADENCE

First time: 10–12 minutes on the lower bench.

Building tolerance: 15 minutes, 2–3 times per week.

Experienced: 2–3 rounds of 15–20 minutes, with cool-downs, 3–4 times per week. The cardiovascular benefits scale with frequency more than duration.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

Pregnancy (consult OB), uncontrolled hypertension, recent heart attack, unstable angina, severe aortic stenosis. Alcohol is a hard no. Hydrate. Cool down between rounds — don't power through. Anyone feeling faint should exit immediately and lie flat. We never let someone push through a wave of nausea or dizziness.

COMMON MEMBER QUESTIONS

Should I pour water on the rocks?

If others in the sauna are okay with it. A small ladle creates a wave of humidity called löyly, which makes the heat feel sharper. Start with a single ladle — it's surprising how much it adds.

Infrared or traditional?

Different feels. Traditional is louder, hotter, more cardiovascular. Infrared is quieter and deeper-warming. Most members use both. Try them in the same week and see what you prefer.

How many rounds?

Two or three is the classic Finnish cycle, with a cool-down between each. If you're new, one round is plenty.

PAIRINGS

The traditional pairing is the **contrast cycle**: sauna → cold plunge → rest → repeat. Two or three rounds, finishing on heat. It's the most-used recovery sequence in the studio for a reason.

04

MODALITY

Compression Therapy.

Sequential pneumatic compression — the “sit and read a book” modality.

WHAT IT IS

Members sit in a recliner while inflatable boots (or sometimes hip/arm sleeves) cycle through pressure patterns from the feet upward. The compression mimics the body's natural muscle pump, helping circulation and lymphatic flow do more in less time. Sessions are typically 20–30 minutes.

THE SCIENCE (THE CREDIBLE VERSION)

Sequential pneumatic compression has decades of clinical use — originally developed to prevent DVT in post-surgical patients. In athletic recovery, it accelerates venous return, enhances lymphatic drainage, and improves clearance of metabolic byproducts from intense exercise. A meta-analysis of recovery studies shows reduced subjective soreness 24–48 hours post-training. For non-athletes, it's an excellent decongestant for tired legs and chronic mild swelling.

WHAT MEMBERS GET

- **Less soreness** the day after hard training.
- **Lighter legs** — especially for desk workers, frequent flyers, and anyone on their feet all day.
- **Genuine rest.** It's the only modality you can fully relax through.
- **Better sleep** when used in the evening, because of the parasympathetic shift.

HOW TO TALK ABOUT IT

MEMBER: *Is this going to hurt?*

YOU: “No. It's firm, like a tight hug that moves up your leg. We start at a moderate pressure and you can ask us to go up or down anytime. Most members close their eyes the first five minutes.”

MEMBER: *Can I do anything while I'm in?*

YOU: “Yes — bring your phone, a book, headphones. Or do nothing. We've had members fall asleep. The boots do the work.”

DURATION & CADENCE

First time: 20 minutes, moderate pressure.

Building: 25–30 minutes, two to three times per week.

Recovery from heavy training: 30 minutes within 24 hours of the session.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

History of DVT or pulmonary embolism, recent vascular surgery, severe peripheral artery disease, open wounds or skin infections on the limb, severe varicose veins, suspected fracture, acute thrombophlebitis. Pregnancy: consult OB. Members with any of the above should consult their physician before using compression.

COMMON MEMBER QUESTIONS

How tight should the boots feel?

Firm but not pinching. If your toes go numb or it feels sharp, tell us and we'll bring the pressure down. The default is dialed for comfort, not endurance.

Will it help my swollen ankles?

It often does — sequential compression supports lymphatic flow. If the swelling is new or only on one side, see a doctor first. We're a recovery tool, not a diagnosis.

How often is too often?

Daily is fine for healthy adults. Many members use it as their wind-down before bed.

PAIRINGS

Compression slots in **anywhere**, which is its superpower. Common pairings: **after sauna** to settle the nervous system, **after a workout** for active recovery, or **as a stand-alone** for tired legs on a busy day.

05

MODALITY

Myofascial Therapy.

Targeted tissue release — the unwinder for what's stuck.

WHAT IT IS

Hands-on and tool-assisted soft tissue work focused on releasing the fascia — the connective tissue web that wraps every muscle. Depending on the member's needs, the session uses guided self-release with foam rollers and percussive tools, or hands-on manual release with the staff guide. Sessions are 15–30 minutes.

THE SCIENCE (THE CREDIBLE VERSION)

Fascia is a continuous, three-dimensional matrix of collagen and ground substance. When tissue is overworked, dehydrated, or held in patterns, fascial layers adhere and slide less freely. Targeted release restores tissue hydration, reduces nociceptor (pain receptor) signaling, and improves range of motion. The effect on pain is partly mechanical, partly neurological — the nervous system “down-regulates” protective tension after a session.

WHAT MEMBERS GET

- **Immediate mobility gains** — range of motion typically improves within the session.
- **Relief from chronic stuck spots** — shoulders, hips, calves, lower back.
- **Better movement quality** the next day in training or daily activity.
- **An education** — members learn what to address at home between sessions.

HOW TO TALK ABOUT IT

MEMBER: *Will it hurt?*

YOU: *“It can be intense, but the rule is “good hurt, not bad hurt.” If you're holding your breath or your whole body's tense, we back off. Pain that makes you brace doesn't release tissue — it just makes it tighter.”*

MEMBER: *Is it like a massage?*

YOU: *“It's in the family, but more targeted. A massage is usually full-body and relaxation-focused. Myofascial work zooms in on specific areas and aims to change how the tissue moves. You'll feel different after, not just relaxed.”*

DURATION & CADENCE

First time: 20 minutes focused on one or two areas.

Regular use: 20–30 minutes, once a week for ongoing tightness.

Acute tightness or pre-event: 15 minutes, targeted.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

Acute injuries (recent strains, sprains, suspected fractures), bone fractures, severe osteoporosis, bleeding disorders, unmanaged blood thinners, recent surgery in the area, deep vein thrombosis, skin infections or open wounds, severe varicose veins. We don't work over active inflammation. If a member describes sharp shooting pain or numbness, refer to a physician.

COMMON MEMBER QUESTIONS

How often should I come?

Once a week is the sweet spot for chronic tightness. More if you're training hard or recovering from a flare-up. Less than every two weeks and you lose the cumulative benefit.

Should I feel sore after?

Mild post-work soreness for 12–24 hours is normal — similar to a workout. Sharp pain, bruising, or anything that gets worse the next day is not. Tell us so we can adjust pressure.

Can I do this myself at home?

Part of the work is teaching you. We'll show you the two or three spots that matter most for your body, and what tools to use between visits.

PAIRINGS

Best **after heat** (infrared or traditional sauna), when tissue is warm and pliable. Often follows compression in a longer session. A good pairing before a **reformer or yoga class** if mobility is limiting movement quality.

06

■■ MODALITY

Reformer Classes.

Spring-loaded Pilates — precision strength, not cardio, not gym lifting.

WHAT IT IS

A small-group class on a Pilates reformer — a sliding carriage with adjustable spring resistance. Members move through controlled, full-range exercises that build the deeper stabilizing muscles. Classes are 50 minutes and led by a certified instructor.

THE SCIENCE (THE CREDIBLE VERSION)

The reformer trains strength through full joint range under controlled tempo and resistance. Unlike free-weight or machine training, it emphasizes eccentric (lengthening) control, postural stabilizers, and breath coordination. Research supports its benefits for chronic low back pain, postural improvement, balance in older adults, and rehabilitation post-injury. It is not a cardiovascular workout in the traditional sense — heart rate stays moderate.

WHAT MEMBERS GET

- **Deep core strength** that supports every other workout they do.
- **Posture improvement** — sometimes visible within weeks.
- **Joint health** — controlled, full-range loading is protective.
- **A movement practice** they can do for life, even with aging or injury.

HOW TO TALK ABOUT IT

MEMBER: *Will it be too easy if I lift weights?*

YOU: *“Most people who lift are surprised. The reformer challenges different muscles — the ones that hold you up rather than the ones that push. It's complementary to lifting, not redundant.”*

MEMBER: *Do I need to be flexible?*

YOU: *“No. The reformer actually supports your body — the springs assist you through ranges you can't reach on your own. Mobility builds with the practice.”*

DURATION & CADENCE

First class: 50 minutes. Arrive 10 minutes early to set up with the instructor.

Building: 2 classes per week is the standard cadence for noticeable strength and posture changes within 6–8 weeks.

Strong base: 3–4 classes per week, or pair reformer with yoga and strength work.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

Late pregnancy requires modifications — refer to a prenatal-trained instructor. Recent surgery, especially abdominal or spinal, needs medical clearance and the instructor needs to know. Active disc herniation, severe osteoporosis, and uncontrolled hypertension require modifications. We always brief the instructor before class on anything the member shares.

COMMON MEMBER QUESTIONS

Will I sweat?

Some. It's not a cardio class but it's not gentle, either. We provide towels for the reformers, so you're covered.

Beginner-friendly?

Yes — every class accommodates beginners. We recommend a one-on-one intro for first-timers to learn the equipment.

How is this different from mat Pilates?

The reformer adds spring resistance and a moving carriage, which lets you load and assist movements in ways mat can't. It's also easier on the wrists and lower back for many people.

PAIRINGS

Reformer pairs beautifully with **yoga** as a complete movement practice. After class, a session of **compression** or **sauna** closes the loop. Members who lift heavy: reformer is the missing antagonist that keeps everything balanced.

07

MODALITY

Yoga & MindBody.

Restorative practice for breath, mobility, and a nervous system that can actually rest.

WHAT IT IS

A range of classes — flow, slow flow, restorative, and breath-led practices — designed to regulate the nervous system, build mobility, and create a moment in the week that isn't striving. Classes are 60 minutes, led by certified instructors.

THE SCIENCE (THE CREDIBLE VERSION)

Slow, breath-led movement and held postures activate the parasympathetic nervous system, lower cortisol, and improve heart rate variability (HRV) — one of the cleanest markers of recovery and stress resilience. Yoga is associated with improvements in chronic pain, anxiety, sleep quality, and balance. Restorative styles in particular shift the body out of sympathetic dominance — the “always-on” state most members are trying to leave.

WHAT MEMBERS GET

- **Stress regulation** — the “come-down” their nervous system has been missing.
- **Mobility** — especially hips, spine, and shoulders.
- **Breath capacity** — useful for sleep, cold plunge, and life in general.
- **Mental quiet** — rare and underrated.

HOW TO TALK ABOUT IT

MEMBER: *I'm not flexible enough for yoga.*

YOU: *“The least flexible people get the most out of it. Flexibility is a result, not a prerequisite. We have classes specifically for stiff bodies — restorative is a great starting point.”*

MEMBER: *I get bored.*

YOU: *“Try the slow flow class — it moves more than restorative without being a workout disguised as yoga. Most members find their attention settles by minute 15.”*

DURATION & CADENCE

First class: 60 minutes. Bring a mat or borrow ours. Arrive 5–10 minutes early.

Building: 2 classes per week is the cadence where most members start feeling the nervous-system shift.

Strong base: 3–4 sessions per week, mixing styles.

SAFETY — WHEN TO SLOW DOWN OR REFER OUT

Pregnancy — refer to a prenatal-trained instructor for any flow class. Glaucoma, severe disc issues, recent surgery, and acute injuries all need modifications. Members new to inverted postures should learn under a teacher's eye. We always tell the instructor what a member shares.

COMMON MEMBER QUESTIONS

Is it hot yoga?

No. Our space is comfortable but not heated. If a member wants hot, we'll point them to a studio nearby — we'd rather they have the right fit.

Will it count as exercise?

Flow classes will get your heart rate up. Restorative classes won't, but they do something equally important — they're how your body learns to recover from everything else you do.

What style do you recommend?

Depends on the week. Stressed and tight: restorative. Stuck and stiff: slow flow. Energized and wanting to move: flow. Try one of each in the first month.

PAIRINGS

Yoga is the perfect **opener** to a recovery session or the **closer** after movement. Pairs especially well with **reformer** for a balanced movement practice, or with **sauna** as a deep weekly reset.

■ ■ PART THREE

Member *moments*.

The four conversations that come up most often — and how we handle them.

■■ MEMBER MOMENT 01

The *first* visit.

Every first-time visitor arrives by one of three paths: they pre-purchased online, they want to buy a day pass or membership on the spot, or they're here for a booked Reformer or Yoga class. All three sign the waiver before anything starts.

BEFORE THEY ARRIVE

- Open the front-desk system. If their name shows in GymMaster (pre-purchase or class booking), check what they paid for and any booking notes — injury, pregnancy, first-time concerns.
- Have a waiver ready — every first-time member signs before any class or modality. No waiver, no session.
- Confirm towels are stocked in the wet and sauna area.
- Know what modalities are currently available so the tour matches reality.

THREE ARRIVAL PATHS: ONE WAIVER, THREE SOPS

Every first-timer signs the waiver. What changes is the order of operations around it. Sort the member into one of the three paths in the first ten seconds of conversation.

Path 1 — Pre-purchased online

They've already paid for a day pass, membership, or class. They'll often say "I bought online" or hand you a confirmation. **Order of operations:** confirm their purchase in GymMaster → sign waiver → tour (if first time) → send them in. Total desk time: 5–7 minutes.

Path 2 — Walk-in (wants to buy on the spot)

They came in cold and want to try the space. **Order of operations:** ask what they're looking for (day pass or membership) → quote the option that fits → process the sale in GymMaster → sign waiver → tour → send them in. Total desk time: 8–12 minutes.

If they're unsure, default to the half-off day pass. Don't push membership at the door — let the experience earn it.

Path 3 — Class Pass Booking

This is the highest-risk SOP, because class passers usually arrive 5–10 minutes before class start and are time-pressured. The waiver gets skipped here more than anywhere else — **don't let it.**

Order of operations: greet at the desk → pull up their class booking in GymMaster to confirm → **hand them the waiver immediately** → while they sign, point them at the bathrooms with showers and the class studio → quick orientation on amenities (towels in wet/sauna area, water station, where to leave shoes). Total desk time: 3–5 minutes — tight, but the waiver is non-negotiable.

If they arrive within 2 minutes of class start without a signed waiver, the instructor cannot put them on a Reformer. Apologize, get the waiver signed, and offer to credit the class. Better to lose one class than skip

the SOP.

THREE PATHS, ONE RULE

Whatever path they're on — pre-purchased, walk-in buyer, or class booking — the waiver is signed before anything else happens. A signed waiver is the first item on the SOP, not the last.

THE ARRIVAL — FIRST 60 SECONDS

Greet by name. Get the waiver signed at the desk first — nothing happens before that's done. Then walk them through the space slowly. Don't recite features — let them see it.

MEMBER: *(Walking in.)*

YOU: “Welcome — glad you made it in. Have you ever done anything like this before?”

YOU: “Before we go anywhere — quick two-minute waiver at the desk. Standard for first visits. Then I'll walk you through the space and we'll figure out what sounds best for today.”

THE TOUR: EVERY MEMBER, EVERY TIME

Every first-time member gets a full walkthrough. Lead them through the space, briefly explain each modality, point out the amenities — then sit them down for one diagnostic question and one recommendation. Two sentences per modality. No pitching.

- **Walk every modality.** Cold plunge, infrared sauna (which is also our red light therapy), traditional sauna, compression, myofascial area, the Reformer studio, and the yoga space. Two sentences each — what it is, what it does.
- **Point out the amenities.** Towels in the wet and sauna area, the 4 bathrooms (2 with showers, 2 without), water station, where to leave their phone and keys.
- **Flag the booking distinction.** Recovery is walk-in any time we're open. Reformer and Yoga need a class booking — show them how to book online.
- **Ask one diagnostic question.** “What does your week usually feel like?” — open enough that what they say tells you what to recommend.
- **Recommend one sequence.** Not a list. One. Based on what you heard.

CLOSING THE TOUR

Wrap the orientation by the bathrooms. Point at where they'll change (the two with showers vs. the two without), the towel station in the wet and sauna area, water, and where to leave their phone, keys, and shoes. Then hand them off to start their session — their pace, on their time.

Check back in once during their session, and again as they're heading out. That brief, unrushed presence is most of what makes the place feel different from another studio.

MEMBER: *(After their session.)*

YOU: “How did that feel?”

YOU: “Most people who come in for a first day pass come back within the first two weeks. If you want, I can hold a Founder spot for you — no commitment, just so you don't lose the rate if you decide to come back.”

■■ MEMBER MOMENT 02

“What's the *difference*?”

Members ask this about modalities, memberships, and us versus other studios. Same answer pattern every time: lead with the goal, not the feature.

MODALITIES — “INFRARED VS. TRADITIONAL SAUNA?”

Don't list specs. Translate to felt experience.

SAMPLE ANSWER

“Traditional sauna is hotter, louder, more cardiovascular. Infrared is quieter, deeper-warming, easier on the airways. If you want the classic post-workout flush, traditional. If you want to sit and decompress, infrared. Most members use both.”

MEMBERSHIPS — “VITAL VS. ULTIMATE?”

Don't pitch the higher tier. Ask before you answer.

SAMPLE ANSWER

“Quick question — are you mostly here for recovery, or do you want to add Reformer and Yoga to the mix? If recovery, Vital's the right answer. If you want unlimited classes, Ultimate. The math usually decides it.”

US VS. ANOTHER STUDIO

Never name the other studio. Speak to what we believe in.

SAMPLE ANSWER

“Every place has its philosophy. Ours is that recovery is the work, not the reward. We built the studio around that. The best way to know if it's a fit is to try a day.”

■■ MEMBER MOMENT 03

“I'm *not* sure.”

They're considering the membership but haven't pulled the trigger. The wrong move is to push. The right move is to remove friction.

WHAT THEY'RE ACTUALLY ASKING

- “Will I use it enough?”
- “What if I commit and life gets busy?”
- “What if I don't like one of the modalities?”

HOW WE RESPOND**SAMPLE ANSWER**

“Three things worth knowing. It's month-to-month — 30 days' notice and you're out. You can freeze it for up to 60 days a year without paying. And the day pass usage usually tells you everything: members who use the studio twice a week within their first two weeks tend to stay for years. Try it, see how it lands.”

IF THEY'RE STILL ON THE FENCE

Hold their Founder rate. Don't pressure them in. The Founder pricing is real and finite, and it makes the conversation about value, not commitment.

MEMBER: *I want to think about it.*

YOU: *“Totally fair. Here's what I can do — I'll hold the Founder rate in your name for two weeks. If you decide yes, the rate is what we talked about. If you decide no, no charge, no pressure.”*

YOU: *“If you have a question between now and then, text the studio number. We'll answer.”*

■■ MEMBER MOMENT 04

Handling *safety concerns*.

A member mentions a condition. Never panic, never dismiss, never play doctor. The rule is simple: ask, respect, refer when in doubt.

THE FOUR QUESTIONS TO ASK

- “What’s your doctor said about heat / cold / pressure / exertion?”
- “Is there a medication you’re on that we should know about?”
- “What have you tolerated well in the past?”
- “Today, do you want to start lower or skip it entirely?”

WHEN TO REFER OUT

Any of these get a “let’s get your physician’s okay first” — and we mean it warmly, not as a brush-off:

- Recent surgery (last 6 weeks).
- Cardiac event in the last 12 months.
- Uncontrolled blood pressure (high or low).
- Pregnancy — first trimester, or any concerns about a current pregnancy.
- Active blood clots, DVT history, or anticoagulant medications.
- Symptoms a member can’t explain (chest pain, persistent dizziness, new swelling).

IF SOMETHING HAPPENS MID-SESSION

Calm voice. Stop the modality. Ask the member to sit. Offer water. Stay with them until they’re stable. Document what happened in the daily log before the end of your shift. When in doubt, call 911 — never gamble with this.

■ ■ PART FOUR

Quick *reference.*

Tables, glossary, and the cheat sheet you can keep behind the desk.

■ QUICK REFERENCE

All modalities at a glance.

MODALITY	FIRST TIME	CADENCE	BEST FOR	WATCH FOR
Cold Plunge	30–60 sec	2–4×/week	Focus, mood, stress tolerance	BP meds, Raynaud's, pregnancy
Infrared Sauna + Red Light	20 min	3–5×/week	Stress, sleep, skin, cardio, joint comfort	Dehydration, BP meds, photosensitizing meds
Traditional Sauna	10–12 min lower bench	3–4×/week	Cardiovascular, decompress, afterglow	Heart conditions, alcohol
Compression	20 min moderate	Daily as needed	Soreness, tired legs, sleep wind-down	DVT history, vascular conditions
Myofascial	20 min, 1–2 areas	1×/week	Chronic tightness, mobility	Acute injury, bleeding disorders
Reformer Classes	50 min, arrive 10 early	2×/week	Core, posture, joint health	Late pregnancy, recent surgery
Yoga & MindBody	60 min restorative	2–3×/week	Nervous system, mobility, breath	Acute injury, glaucoma, late pregnancy

■ ■ REFERENCE

Glossary.

The vocabulary you'll hear from members and what each term actually means.

Cold shock response

The involuntary gasp and rapid breathing when you first enter cold water. Lasts about 30 seconds. Controlled by slow nasal exhales.

Cytochrome c oxidase

The enzyme inside mitochondria that absorbs red and near-infrared light. The biochemical reason red light therapy works.

Diving reflex

A mammalian reflex that activates when the face hits cold water — slows heart rate, conserves oxygen. Why face-dunking calms the nervous system.

Fascia

Connective tissue that wraps every muscle, organ, and bone. The target of myofascial work.

Heart rate variability (HRV)

The variation in time between heartbeats. Higher HRV generally means better recovery and a more adaptive nervous system.

Heat shock proteins (HSP)

Proteins produced under heat stress that help cells repair and resist future stress. Part of why sauna use compounds over time.

Hormetic stress

A small dose of stress that triggers an adaptive response — the body comes back stronger. Cold plunge, sauna, and exercise all work this way.

Löyly

Finnish term for the wave of humidity and heat that follows water on the sauna stones. Pronounced “LOY-loo.”

Parasympathetic nervous system

The “rest and digest” branch — slows heart rate, promotes digestion, supports recovery. The system most members are trying to access.

Photobiomodulation

The technical term for what red light therapy does — modulating cell function through specific light wavelengths.

Sympathetic nervous system

The “fight or flight” branch — raises heart rate, sharpens focus, mobilizes energy. Activated briefly by cold; should not be the default state.

Vagal tone

The activity of the vagus nerve, the main highway of the parasympathetic system. Strengthened by slow breathing, cold-water face immersion, and meditation.



Recover smarter.
Live stronger.

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